

SUPERVISED VOCATIONAL QUALIFICATION DIPLOMA - EXAMINATION

Session: Revision Mock Set 04	Duration: 3 hours
Specialty: Psychotherapy	Coefficient: 04
Eliminatory mark: < 8/20	Training reference: VTI-AEDA examination style

COMPREHENSIVE VOCATIONAL PAPER - SOLUTION / MARKING GUIDE PRACTICE SET 04

Revision material based on the supplied VTI-AEDA 2025 examination format and the supplied psychotherapy course recordings. Not an official examination paper.

How to use this marking guide

For revision, first attempt the paper without notes. Then compare not only the answer letter but the clinical cue that makes the answer correct. For legal/ethical items, the guide follows the concepts emphasized by the supplied course/examination materials; in real practice, applicable Cameroon law, institutional policy and the current professional code must also be checked.

PART ONE: PSYCHOPATHOLOGY - ANSWERS

1. During assessment, Joel reports after trauma, recurrent intrusions or nightmares, avoidance, negative changes in mood/cognition and heightened arousal persisting beyond one month. Which of the following is the BEST match?

A. Post-traumatic stress disorder (PTSD)

B. Agoraphobia

C. Schizophrenia

D. Opioid intoxication

Answer: A - Post-traumatic stress disorder (PTSD). PTSD follows exposure to trauma and is organized around intrusion, avoidance, negative cognitive/mood changes and arousal/reactivity.

Memory aid: IANA: *Intrusion, Avoidance, Negative changes, Arousal.*

2. During assessment, Yvette reports at least two weeks of depressed mood and/or loss of interest with associated sleep, appetite, energy, concentration, guilt, psychomotor or suicidal symptoms. Which of the following is the BEST match?

A. Social anxiety disorder

B. Bipolar disorder

C. Brief psychotic disorder

D. Major depressive disorder

Answer: D - Major depressive disorder. Major depression is a sustained depressive syndrome, not simply transient sadness. Functional impairment and risk, especially suicidality, must be assessed.

Memory aid: SIGECAPS: *Sleep, Interest, Guilt, Energy, Concentration, Appetite, Suicide.*

3. Lucas, a 58-year-old farmer, presents with abnormally elevated or irritable mood with increased energy, reduced need for sleep, pressured speech, grandiosity, racing thoughts and risky activity. Which diagnosis or clinical concept best fits this presentation?

A. Autism spectrum disorder

B. Manic episode

C. Schizoid personality disorder

D. Substance use disorder

Answer: B - Manic episode. A manic episode combines abnormal mood elevation/irritability with increased energy and characteristic activation symptoms.

Memory aid: DIG FAST: *Distractible, Indiscretion, Grandiosity, Flight of ideas, Activity, Sleep less, Talkative.*

4. Yvette, a 59-year-old mechanic, presents with a history of a manic episode, often alternating with periods of depression. Which diagnosis or clinical concept best fits this presentation?

A. Bipolar disorder

- B. Obsessive-compulsive disorder (OCD)
- C. Manic episode
- D. Anorexia nervosa, binge-eating/purging type

Answer: A - Bipolar disorder. The defining feature of bipolar I disorder is a manic episode; depressive episodes are common but not required for that diagnosis.

Memory aid: *BI-polar = mood can move toward two poles; mania is the key flag.*

5. Patricia, a 56-year-old teacher, presents with persistent psychotic illness with delusions, hallucinations and/or disorganized speech or behavior, often with social or occupational dysfunction. Which diagnosis or clinical concept best fits this presentation?

- A. Brief psychotic disorder
- B. Schizophrenia**
- C. Autism spectrum disorder
- D. Opioid intoxication

Answer: B - Schizophrenia. Schizophrenia is a chronic psychotic disorder; diagnosis depends on characteristic symptoms, duration and functional impact, not on a laboratory test alone.

Memory aid: *3D psychosis: Delusions, perceptual Disturbance, Disorganization.*

6. A therapist notes that Hassan has sudden onset of delusions, hallucinations or disorganized speech/behavior lasting at least a day but less than a month, with return toward baseline. Which condition should be considered first from the options below?

- A. Brief psychotic disorder**
- B. Substance use disorder
- C. Agoraphobia
- D. Social anxiety disorder

Answer: A - Brief psychotic disorder. The very short duration and return toward premorbid functioning distinguish brief psychotic disorder from longer psychotic disorders.

Memory aid: *BRIEF = psychosis Below one month.*

7. During assessment, Esther reports significantly low body weight, intense fear of weight gain and restriction without recurrent binge-purge episodes. Which of the following is the BEST match?

- A. Obsessive-compulsive personality disorder (OCPD)
- B. Generalized anxiety disorder (GAD)
- C. Post-traumatic stress disorder (PTSD)
- D. Anorexia nervosa, restricting type**

Answer: D - Anorexia nervosa, restricting type. Anorexia nervosa requires significantly low weight plus fear of gaining weight or persistent behavior interfering with weight gain and body-image disturbance.

Memory aid: *ANOREXIA: restriction + low weight + fear of weight gain.*

8. A therapist notes that Fidel has significantly low body weight with anorexia features plus recurrent binge-eating or purging. Which condition should be considered first from the options below?

- A. Stimulant intoxication
- B. Bulimia nervosa
- C. Anorexia nervosa, binge-eating/purging type**
- D. Agoraphobia

Answer: C - Anorexia nervosa, binge-eating/purging type. The patient meets anorexia criteria including low weight, but also has recurrent binge/purge behavior.

Memory aid: *Low weight first: if anorexia criteria are met, binge/purge does not automatically mean bulimia.*

9. Grace, a 27-year-old teacher, presents with recurrent binge eating followed by compensatory behaviors such as vomiting, laxatives or excessive exercise, without the persistently low weight required for anorexia. Which diagnosis or clinical concept best fits this presentation?

- A. Anorexia nervosa, restricting type
- B. Bulimia nervosa**

C. Substance use disorder

D. Opioid intoxication

Answer: B - Bulimia nervosa. Bulimia is defined by binge episodes and inappropriate compensation; weight may be normal or above/below normal but not at the markedly low anorexia threshold.

Memory aid: BULIMIA = Binge + Undo.

10. A therapist notes that Victor has recurrent binge eating with loss of control and distress but without regular compensatory purging or fasting. Which condition should be considered first from the options below?

A. Obsessive-compulsive disorder (OCD)

B. Binge-eating disorder

C. Opioid intoxication

D. Borderline personality disorder

Answer: B - Binge-eating disorder. Binge-eating disorder differs from bulimia because recurrent compensatory behaviors are absent.

Memory aid: BED = Binge, no regular Erasing behavior.

SECTION B: TRUE/FALSE - EXPLANATIONS

1. FALSE. PTSD is diagnosed solely because a frightening event occurred.

Trauma exposure is necessary but not sufficient; a characteristic symptom pattern and duration/impact are also required.

2. TRUE. Dependent personality disorder may involve difficulty making everyday decisions without reassurance.

Excessive reliance on others is a central feature.

3. TRUE. An opioid is more likely than a stimulant to produce drowsiness and physiologic slowing.

Opioids commonly produce sedation, whereas stimulants activate.

4. FALSE. A manic episode is simply a happy mood with no change in energy or functioning.

Mania involves abnormal mood plus increased energy/activity and additional symptoms with significant impact.

5. TRUE. Anorexia nervosa can include binge-eating/purging behavior if the person also meets the low-weight anorexia criteria.

That is the binge-eating/purging subtype of anorexia nervosa.

SECTION C: MODEL SHORT ANSWERS

1. Differentiate a panic attack from panic disorder. (2mks)

- A panic attack is an acute episode of intense fear/discomfort with physical/cognitive symptoms.

- Panic disorder requires recurrent unexpected attacks plus persistent concern and/or maladaptive behavior change.

Memory aid: Attack = episode; disorder = recurring pattern + aftermath.

2. Differentiate obsession from compulsion and give one example of each. (2mks)

- Obsession: recurrent intrusive unwanted thought/urge/image, e.g. fear of contamination.

- Compulsion: repetitive behavior/mental act to reduce distress, e.g. repeated hand washing.

Memory aid: O = thought; C = conduct.

3. State the four D criteria commonly used when considering abnormality. (2mks)

- Deviance

- Distress

- Dysfunction

- Danger

Memory aid: 4D: Deviance, Distress, Dysfunction, Danger.

4. Define substance use disorder in practical clinical terms. (2mks)

- Problematic pattern of substance use causing significant impairment or distress.

- Look for loss of control, social/role impairment, risky use and/or tolerance/withdrawal.

Memory aid: 4C: Control loss, Consequences, Craving, Continued use.

5. Give two reasons early intervention in substance use disorder is important. **(2mks)**

- Reduces medical, social, occupational and legal harm and risk of escalation.
- Improves opportunity for engagement, relapse prevention, family support and recovery before consequences accumulate.

Memory aid: *EARLY = less Entrenchment, Address Risk, Link help, Yield recovery.*

PART TWO: THEORIES/TECHNIQUES - ANSWERS

1. A therapist working with Tina uses an intervention characterized by an approach emphasizing freedom, responsibility, choice, meaning, authenticity, isolation and awareness of mortality. Which approach or technique is being used?

- A. Countertransference
- B. Free association
- C. Existential therapy**
- D. Social skills training

Answer: C - Existential therapy. Existential therapy helps clients face givens of existence and make authentic choices rather than simply trying to eliminate all anxiety.

Memory aid: FMRC: Freedom, Meaning, Responsibility, Choice.

2. A therapist working with Victor uses an intervention characterized by learning by association between stimuli so that a previously neutral cue comes to elicit a response. Which approach or technique is being used?

- A. Classical conditioning**
- B. Countertransference
- C. Systematic desensitization
- D. Resistance

Answer: A - Classical conditioning. Classical conditioning explains learned associations such as fear conditioning and underlies some exposure-based principles.

Memory aid: Classical = cue-to-cue association.

3. In a session with Yvette, the counselor emphasizes behavior is shaped by its consequences, including reinforcement and punishment. This BEST illustrates:

- A. Adlerian therapy
- B. Operant conditioning**
- C. Social modeling
- D. In vivo exposure

Answer: B - Operant conditioning. Operant conditioning focuses on consequences that increase or decrease behavior.

Memory aid: Operant = behavior OPERATES, consequence follows.

4. Which psychotherapy concept is most accurately described by the following: a behavior increases because it removes or prevents an aversive stimulus?

- A. Negative reinforcement**
- B. Progressive muscle relaxation
- C. Resistance
- D. Operant conditioning

Answer: A - Negative reinforcement. Negative reinforcement strengthens behavior; negative means something unpleasant is removed, not that behavior is punished.

Memory aid: Reinforcement always raises behavior; negative removes something.

5. Which psychotherapy concept is most accurately described by the following: a previously reinforced behavior decreases when the reinforcement maintaining it is consistently withheld?

- A. Operant conditioning
- B. Extinction**
- C. Classical conditioning
- D. Gestalt dream work

Answer: B - Extinction. Extinction occurs when reinforcement is no longer provided for a behavior.

Memory aid: Extinction = extinguish the payoff.

6. A therapist working with Wendy uses an intervention characterized by learning behavior by observing what happens to another person or by imitating a model. Which approach or technique is being used?

- A. Social skills training
- B. Gestalt dream work
- C. Countertransference

D. Social modeling

Answer: D - Social modeling. Modeling is observational learning: the client learns from another person's behavior and its consequences.

Memory aid: MODEL = learn by watching a model.

7. A therapist working with Owen uses an intervention characterized by planned contact with feared stimuli, memories or situations while reducing avoidance so fear can be relearned. Which approach or technique is being used?

- A. Rational emotive imagery
- B. Extinction
- C. Flooding

D. Exposure therapy

Answer: D - Exposure therapy. Exposure reduces avoidance and allows corrective learning; it may be in vivo, imaginal or other formats.

Memory aid: Approach, do not avoid.

8. In a session with Hassan, the counselor emphasizes direct, real-life exposure to a feared object or situation. This BEST illustrates:

- A. Classical conditioning
- B. Social modeling
- C. In vivo exposure**
- D. Empty-chair technique

Answer: C - In vivo exposure. In vivo means exposure in the actual situation rather than only in imagination.

Memory aid: VIVO = in real life.

9. Which psychotherapy concept is most accurately described by the following: gradual exposure to a fear hierarchy combined with relaxation or another incompatible response?

- A. Systematic desensitization**
- B. Transference
- C. Gestalt dream work
- D. Cognitive behavioral therapy (CBT)

Answer: A - Systematic desensitization. Systematic desensitization classically combines relaxation training with stepwise movement through a fear hierarchy.

Memory aid: RHE: Relax, Hierarchy, Expose gradually.

10. In a session with Uma, the counselor emphasizes rapid and prolonged exposure to a high-intensity feared stimulus or situation without gradual hierarchy progression. This BEST illustrates:

- A. Social modeling
- B. Person-centered therapy
- C. Negative reinforcement

D. Flooding

Answer: D - Flooding. Flooding differs from graded exposure because the client confronts a high level of fear more rapidly.

Memory aid: Flooding = jump into the deep end.

11. In a session with Grace, the counselor emphasizes systematically tensing and releasing muscle groups to learn physical relaxation and reduce arousal. This BEST illustrates:

- A. Operant conditioning

- B. Classical conditioning
- C. Cognitive behavioral therapy (CBT)

D. Progressive muscle relaxation

Answer: D - Progressive muscle relaxation. PMR builds awareness of tension and relaxation by working through muscle groups.

Memory aid: *Tense - notice - release - breathe.*

12. A therapist working with Wendy uses an intervention characterized by a skills-based therapy especially associated with emotion dysregulation and borderline personality disorder, using mindfulness, distress tolerance, emotion regulation and interpersonal effectiveness. Which approach or technique is being used?

A. Dialectical Behavior Therapy (DBT)

- B. Flooding
- C. Rational emotive imagery
- D. Transference

Answer: A - Dialectical Behavior Therapy (DBT). DBT combines acceptance and change strategies and teaches concrete skills for severe emotion dysregulation.

Memory aid: *M-D-E-I: Mindfulness, Distress tolerance, Emotion regulation, Interpersonal effectiveness.*

13. In a session with Xavier, the counselor emphasizes behavioral teaching and rehearsal of interpersonal skills such as greetings, eye contact, listening and conversation. This BEST illustrates:

- A. Rational emotive imagery
- B. Interpretation
- C. Social skills training**
- D. Social modeling

Answer: C - Social skills training. Social skills training uses instruction, modeling, rehearsal and feedback to build interpersonal behavior.

Memory aid: *IMRF: Instruction, Modeling, Rehearsal, Feedback.*

14. A therapist working with Lucas uses an intervention characterized by the client says whatever thoughts, feelings, images or memories come to mind without censoring them. Which approach or technique is being used?

- A. Resistance
- B. Free association**
- C. In vivo exposure
- D. Interpretation

Answer: B - Free association. Free association is a classic psychoanalytic technique intended to reduce censorship and reveal unconscious material.

Memory aid: *Psychoanalytic FIRT: Free association, Interpretation, Resistance, Transference.*

15. Which psychotherapy concept is most accurately described by the following: the therapist explains or offers meaning about patterns, defenses, dreams, resistance or behavior?

- A. Classical conditioning
- B. Interpretation**
- C. Social skills training
- D. Dialectical Behavior Therapy (DBT)

Answer: B - Interpretation. Interpretation is the analyst's attempt to make latent or previously unconscious meaning more understandable to the client.

Memory aid: *Interpretation = therapist offers possible meaning.*

SECTION B: TRUE/FALSE - EXPLANATIONS

1. FALSE. Progressive muscle relaxation involves systematically increasing tension and never releasing it.

The method alternates tension with deliberate release and relaxation.

2. **FALSE.** In vivo exposure occurs only in imagination.

In vivo exposure takes place in the real feared situation or with the real feared object.

3. **TRUE.** Retroflexion involves directing toward oneself an impulse that might otherwise be directed outward.

That is the Gestalt meaning of retroflexion.

4. **TRUE.** Rational emotive imagery belongs to the REBT family of techniques.

It is used to rehearse changing unhealthy emotion by changing beliefs.

5. **TRUE.** Social skills training can use modeling, role play, rehearsal and feedback.

Those are common behavioral teaching components.

SECTION C: MODEL SHORT ANSWERS

1. State one technique specific to psychoanalytic work. **(2mks)**

- Free association, interpretation, analysis of resistance, transference analysis or dream analysis - any one.

Memory aid: *FIRT*.

2. Explain systematic desensitization in three steps. **(2mks)**

- Teach relaxation/coping response.

- Build a graded fear hierarchy.

- While relaxed, expose progressively from easier to harder items until anxiety reduces.

Memory aid: *RHE*.

3. Distinguish flooding from graded exposure. **(1mks)**

- Flooding begins with high-intensity feared material and sustains exposure.

- Graded exposure moves stepwise from lower to higher fear levels.

Memory aid: *Flooding = deep end; graded = ladder*.

PART THREE: ETHICAL/LEGAL - ANSWERS

1. A counselor asks which principle or professional concept is defined by maintaining accurate, timely, secure and clinically appropriate client records. The BEST answer is:

- A. Record keeping**
- B. Mandated reporting
- C. Boundary violation
- D. Gift decision-making

Answer: A - Record keeping. Records support continuity of care, accountability and legal/ethical duties; access must be controlled and retention rules followed.

Memory aid: ACTS: Accurate, Current, Timely, Secure.

2. A counselor asks which principle or professional concept is defined by ending therapy responsibly with preparation, continuity planning and referral when needed rather than abandoning the client. The BEST answer is:

- A. Boundary crossing
- B. Termination and referral**
- C. Boundary violation
- D. Bartering

Answer: B - Termination and referral. Ethical termination includes adequate notice, discussion, referrals or continuity plans when appropriate.

Memory aid: PLAN: Prepare, Link/referral, Address feelings, Next steps.

3. A counselor asks which principle or professional concept is defined by using ethical codes, consultation/supervision and appropriate reporting procedures when credible serious misconduct is observed. The BEST answer is:

- A. Beneficence
- B. Informed consent
- C. Ethical response to colleague misconduct**
- D. Termination and referral

Answer: C - Ethical response to colleague misconduct. Serious ethical violations should not simply be ignored; follow code, organizational and legal procedures while protecting clients and due process.

Memory aid: C-A-R-E: Consult code, Assess facts, Report appropriately, Ensure client safety.

4. A counselor asks which principle or professional concept is defined by protecting privacy, competence, informed consent, identity, emergency planning and data security when using digital or remote services. The BEST answer is:

- A. Termination and referral
- B. Technology ethics**
- C. Beneficence
- D. Professional competence

Answer: B - Technology ethics. Technology changes the medium, not the professional duty. Security, consent and contingency planning remain essential.

Memory aid: SPACE: Security, Privacy, Access/identity, Consent, Emergency plan.

5. A counselor asks which principle or professional concept is defined by not automatically disclosing the entire file on receipt of a subpoena; consult applicable law, the client/attorney and professional guidance, and disclose only as authorized or legally required. The BEST answer is:

- A. Sexual relationship with a current client
- B. Autonomy
- C. Subpoena response**
- D. Ethical response to colleague misconduct

Answer: C - Subpoena response. A subpoena and a court order are not always identical. The safest course-based principle is not to reflexively release all records; seek proper legal/ethical guidance and protect confidentiality to the extent permitted.

Memory aid: STOP: See legal guidance, Tell/consult client, Only required information, Protect confidentiality.

6. During supervision, Kemi describes respecting the client's right to make informed choices about treatment. Which concept should guide the discussion?

- A. Termination and referral
- B. Technology ethics
- C. Beneficence

D. Autonomy

Answer: D - Autonomy. Autonomy supports informed choice, consent and respect for the client's self-determination.

Memory aid: A = Allow informed choice.

7. In professional practice, a situation calls for acting to promote the client's welfare and benefit. Which ethical/legal concept is MOST directly involved?

- A. Duty to warn/protect
- B. Beneficence**
- C. Professional competence
- D. Mandated reporting

Answer: B - Beneficence. Beneficence asks the professional to actively contribute to the client's well-being.

Memory aid: BENEFIT = beneficence.

8. A counselor asks which principle or professional concept is defined by avoiding or minimizing harm. The BEST answer is:

- A. Professional competence
- B. Nonmaleficence**
- C. Boundary crossing
- D. Justice

Answer: B - Nonmaleficence. Nonmaleficence is the duty to do no harm and to avoid exposing clients to unnecessary risk.

Memory aid: NON-maleficence = NO harm.

9. During supervision, Zachary describes fair and equitable distribution of services, benefits, burdens and opportunities. Which concept should guide the discussion?

- A. Boundary crossing
- B. Fidelity
- C. Gift decision-making

D. Justice

Answer: D - Justice. Justice concerns fairness and equitable treatment, including access and resource allocation.

Memory aid: Justice = fairness.

10. A counselor asks which principle or professional concept is defined by truthfulness and honest communication with clients. The BEST answer is:

- A. Subpoena response
- B. Ethical response to colleague misconduct
- C. Veracity**
- D. Boundary crossing

Answer: C - Veracity. Veracity requires accurate, truthful information rather than deception.

Memory aid: VERACITY = verifiable truth.

11. During supervision, Patricia describes keeping commitments, maintaining trust and being faithful to professional responsibilities. Which concept should guide the discussion?

- A. Informed consent
- B. Bartering

C. Gift decision-making

D. Fidelity

Answer: D - Fidelity. Fidelity concerns loyalty, promises and trust within professional relationships.

Memory aid: *Fidelity = faithful to promises.*

12. A counselor asks which principle or professional concept is defined by an ongoing process in which the client receives understandable information about services, risks, benefits, alternatives, limits of confidentiality and choice. The BEST answer is:

- A. Professional competence
- B. Subpoena response
- C. Duty to warn/protect

D. Informed consent

Answer: D - Informed consent. Informed consent is not merely a signature; it is an ongoing communication process supporting voluntary, informed participation.

Memory aid: *BRALC: Benefits, Risks, Alternatives, Limits, Choice.*

13. A counselor asks which principle or professional concept is defined by the professional duty to protect information learned in the therapeutic relationship, subject to recognized legal and ethical exceptions. The BEST answer is:

- A. Boundary violation
- B. Confidentiality**
- C. Autonomy
- D. Professional competence

Answer: B - Confidentiality. Confidentiality protects client information but is not absolute; limits should be explained early.

Memory aid: *Confidentiality = keep private, know the exceptions.*

14. A counselor asks which principle or professional concept is defined by the client's broader right to control access to self, body and personal information. The BEST answer is:

- A. Nonmaleficence
- B. Boundary violation
- C. Duty to warn/protect

D. Privacy

Answer: D - Privacy. Privacy is the client's right to be left alone or control access, whereas confidentiality is the professional's duty regarding information received.

Memory aid: *Privacy belongs to the person; confidentiality binds the professional.*

15. In professional practice, a situation calls for a legal protection that may prevent compelled disclosure of certain therapy communications in judicial proceedings, depending on jurisdiction. Which ethical/legal concept is MOST directly involved?

- A. Privileged communication**
- B. Boundary violation
- C. Justice
- D. Technology ethics

Answer: A - Privileged communication. Privilege is a legal evidentiary concept and is distinct from the broader ethical duty of confidentiality.

Memory aid: *Privilege = courtroom/legal shield.*

SECTION B: MODEL SHORT ANSWERS

1. Define malpractice. (2mks)

- Professional negligence in which a duty is breached and the breach causes harm; commonly considered through duty, breach, causation and damages/harm.

Memory aid: *DBHC.*

2. Give two elements of informed consent. (1mks)

- Examples: nature/purpose of services, risks, benefits, alternatives, fees, therapist qualifications, limits of confidentiality, voluntary choice, right to ask questions.

Memory aid: BRALC.

3. When is termination ethically appropriate? (1mks)

- When goals are met, treatment is no longer beneficial/needed, the therapist cannot provide competent service, or other legitimate reasons exist - with adequate planning to avoid abandonment.

Memory aid: PLAN.

4. State one key ethical requirement for clinical records. (1mks)

- Records should be accurate, timely, relevant and securely stored with access protected.

Memory aid: ACTS.

SECTION C: MODEL SHORT ESSAYS

1. Why are dual relationships ethically risky? (2mks)

- They can impair objectivity or professional judgment.
- They can increase risk of exploitation, coercion, conflicts of interest or harm.

Memory aid: Power + objectivity + exploitation.

2. What is a boundary violation? (2mks)

- A serious harmful or exploitative breach of therapeutic boundaries.

Memory aid: Violation = harm/exploitation.

3. State two technology-related ethical concerns in teletherapy. (1mks)

- Examples: data security/privacy, informed consent, identity verification, competence with technology, emergency location/plan, jurisdiction.

Memory aid: SPACE.

PART FOUR: CULTURE / GROUP / COUNSELING PROCESS / TREATMENT PLAN - ANSWERS

1. Which counseling/group/cultural concept is BEST described by: systematically exploring the client's problems, symptoms, context, history, strengths, risk and functioning?

- A. Storming stage
- B. Norming stage
- C. Termination and follow-up

D. Assessment

Answer: D - Assessment. Assessment precedes or accompanies formulation and goal setting; it includes both problems and resources.

Memory aid: HRSF: History, Risk, Symptoms, Functioning.

2. In counseling practice, Wendy encounters a situation involving collaboratively defining client-centered outcomes that are realistic and observable. Which concept BEST applies?

- A. Goal setting**
- B. Adjourning stage
- C. Group confidentiality
- D. Skills-development group

Answer: A - Goal setting. Goals are stronger when the client participates and the goals are specific enough to evaluate.

Memory aid: SMART: Specific, Measurable, Achievable, Realistic/Relevant, Time-bound.

3. Which counseling/group/cultural concept is BEST described by: implementing agreed therapeutic strategies and monitoring the client's response?

- A. Termination and follow-up
- B. Active listening
- C. Performing stage

D. Intervention stage

Answer: D - Intervention stage. Intervention follows assessment and goal definition and should be linked to the case formulation and treatment plan.

Memory aid: Assess -> Goal -> Intervene -> Review.

4. Which counseling/group/cultural concept is BEST described by: reviewing progress, consolidating coping skills, planning relapse prevention or referral and ending the professional relationship responsibly?

- A. Assessment
- B. Cultural imposition
- C. Performing stage

D. Termination and follow-up

Answer: D - Termination and follow-up. Termination is a therapeutic phase, not simply stopping appointments.

Memory aid: RECAP: Review, Evaluate, Consolidate, Aftercare, Part.

5. In counseling practice, Kemi encounters a situation involving attending closely to verbal and nonverbal communication and showing the client that the message has been heard and understood. Which concept BEST applies?

- A. Active listening**
- B. Assessment
- C. Open-ended questioning
- D. Therapeutic confrontation

Answer: A - Active listening. Active listening includes presence, minimal encouragers, reflection, clarification and appropriate nonverbal attending.

Memory aid: SOLER: Squarely face, Open posture, Lean, Eye contact, Relaxed.

6. In counseling practice, Wendy encounters a situation involving identifying and feeding back the emotional meaning expressed by the client. Which concept BEST applies?

- A. Psychoeducational group
- B. Active listening
- C. Paraphrasing

D. Reflection of feeling

Answer: D - Reflection of feeling. Reflection of feeling helps clients recognize and explore emotions rather than only facts.

Memory aid: *Name the feeling, not just the facts.*

7. A trainee observes restating the essence of the client's content in fresh words without changing its meaning. This is an example of:

- A. Paraphrasing**
- B. Case formulation
- C. Performing stage
- D. Self-disclosure

Answer: A - Paraphrasing. Paraphrasing checks understanding and encourages elaboration.

Memory aid: *Paraphrase = same meaning, new words.*

8. Which counseling/group/cultural concept is BEST described by: questions that invite elaboration rather than a yes/no answer?

- A. Open-ended questioning**
- B. Assessment
- C. Paraphrasing
- D. Forming stage

Answer: A - Open-ended questioning. Open questions support exploration, while excessive questioning can turn therapy into an interrogation.

Memory aid: *WHAT/HOW usually open more than DID/IS.*

9. In counseling practice, Hassan encounters a situation involving respectfully pointing out discrepancies between a client's words, behaviors, values or nonverbal cues. Which concept BEST applies?

- A. Group role
- B. Therapeutic confrontation**
- C. Case formulation
- D. Paraphrasing

Answer: B - Therapeutic confrontation. Confrontation is not aggression; it is a careful invitation to examine inconsistency.

Memory aid: *Confront discrepancy, not the person.*

10. Which counseling/group/cultural concept is BEST described by: the therapist deliberately shares limited personal information when it is likely to benefit the client rather than meet the therapist's own needs?

- A. Reflection of feeling
- B. Self-disclosure**
- C. Active listening
- D. Cognitive-behavioral group

Answer: B - Self-disclosure. Self-disclosure should be purposeful, brief and client-centered.

Memory aid: *WAIT: Why am I telling this?*

11. A trainee observes a collaborative written or explicit plan linking identified problems to goals, measurable objectives, interventions and review. This is an example of:

- A. Performing stage
- B. Cognitive-behavioral group
- C. Treatment plan**
- D. Skills-development group

Answer: C - Treatment plan. Treatment plans organize care around the client's specific needs and should be revised according to progress.

Memory aid: *PGOIR: Problem, Goal, Objectives, Interventions, Review.*

12. A trainee observes an organized explanation linking the client's presenting problems with relevant history, triggers, maintaining factors, strengths and a rationale for intervention. This is an example of:

- A. Case formulation
- B. Adjourning stage
- C. Termination and follow-up
- D. Therapeutic confrontation

Answer: A - Case formulation. Formulation goes beyond a diagnosis by explaining why this person is struggling in this way at this time and what may help.

Memory aid: *5P option: Presenting, Predisposing, Precipitating, Perpetuating, Protective.*

13. A trainee observes evaluating whether objectives are being achieved and modifying the treatment plan when progress is absent, circumstances change or goals are met. This is an example of:

- A. Cultural imposition
- B. Group role
- C. Progress review
- D. Ethnocentrism

Answer: C - Progress review. Treatment planning is dynamic; review links evidence of progress to continuation, modification or termination.

Memory aid: *Review = measure, discuss, revise.*

14. In counseling practice, Uma encounters a situation involving ongoing development of awareness, cultural knowledge, clinical skills and meaningful encounters with diverse clients. Which concept BEST applies?

- A. Assessment
- B. Cultural competence
- C. Active listening
- D. Cognitive-behavioral group

Answer: B - Cultural competence. Cultural competence requires reflective awareness plus knowledge, skills and real encounters; it is not achieved by assuming everyone should conform to the therapist's culture.

Memory aid: *AKSE: Awareness, Knowledge, Skills, Encounters.*

15. In counseling practice, Patricia encounters a situation involving judging other cultures primarily by the standards of one's own culture and treating one's own as the norm. Which concept BEST applies?

- A. Group role
- B. Therapeutic confrontation
- C. Ethnocentrism
- D. Group confidentiality

Answer: C - Ethnocentrism. Ethnocentrism places one's own cultural frame at the center and may distort assessment or communication.

Memory aid: *ETHNO-centred = my culture at the centre.*

SECTION B: MODEL SHORT ESSAYS

1. What is case formulation? (4mks)

- An organized explanation of how presenting problems connect with history, triggers, maintaining factors and strengths/protective factors.
- It guides treatment by explaining why the problem developed/persists and what interventions make sense.

Memory aid: *5P is one useful organizing mnemonic.*

2. Describe the storming and norming stages of group development. (4mks)

- Storming: conflict, resistance, competition and testing of roles/leader.
- Norming: cohesion, shared norms, trust and cooperation become more stable.

Memory aid: *Storm -> Norm.*

3. Give four examples of counseling skills used in an intake session. **(2mks)**

- Examples: active listening, empathy, open-ended questions, reflection, paraphrasing, summarizing, clarification, therapeutic silence, appropriate confrontation.

Memory aid: *LEAP: Listen, Empathize, Ask, Paraphrase.*

SUPERVISED VOCATIONAL QUALIFICATION DIPLOMA - EXAMINATION

Session: Revision Mock Set 04	Duration: 1hr 30mins
Specialty: Psychotherapy	Coefficient: 04
Eliminatory mark: < 14/20	Training reference: VTI-AEDA examination style

SIMULATION OF REAL WORKING SITUATION PAPER - SOLUTION / MARKING GUIDE PRACTICE SET 04

Revision material based on the supplied VTI-AEDA 2025 examination format and the supplied psychotherapy course recordings. Not an official examination paper.

DETAILED MODEL ANSWERS - SIMULATION OF REAL WORKING SITUATIONS

SECTION I

1. Emotion dysregulation and self-harm - anchor: Borderline personality features - DBT-informed treatment with safety planning

Clara, 19, describes rapidly shifting intense relationships, fear of abandonment, impulsive spending, recurrent self-cutting and episodes of overwhelming anger followed by shame.

Model response / marking points (25 marks):

1. Assess current self-harm/suicide risk first and establish a safety/crisis plan.
2. Validate suffering while maintaining clear boundaries.
3. Explain the DBT balance of acceptance and change.
4. Teach mindfulness of current emotion/urge.
5. Introduce distress-tolerance alternatives to self-harm.
6. Teach emotion-regulation skills and trigger tracking.
7. Address interpersonal effectiveness and abandonment-related behaviors.
8. Use behavioral chain analysis around a recent self-harm episode.
9. Set treatment hierarchy and collaborative goals.
10. Coordinate psychiatric/medical care when indicated.

Suggested mark allocation: Award marks for correct formulation/approach, risk and ethical priorities, logical session structure, appropriate technique application, collaboration/homework/follow-up, and professional communication. Credit clinically sound alternatives when clearly justified. Do not award full marks for naming a theory without showing how it is applied to the facts of the vignette.

Memory aid: MDEI: Mindfulness, Distress tolerance, Emotion regulation, Interpersonal effectiveness.

2. Authority, guilt and independence - anchor: Existential and/or Adlerian formulation, with assertiveness practice

Emmanuel, 26, grew up in a strict family and school environment. he automatically seeks approval from authority figures, feels guilty when disagreeing, and wants to make independent adult choices but fears rejection and moral failure.

Model response / marking points (25 marks):

1. Build an accepting relationship and explore the client's lived experience.
2. Clarify inherited values versus personally chosen values without imposing the therapist's values.
3. Explore freedom, responsibility and anxiety that accompany choice.
4. Identify life goals and patterns of approval-seeking.
5. Adlerian option: examine lifestyle/private logic and use encouragement toward social interest and responsible choice.
6. Existential option: explore authenticity, meaning and willingness to tolerate guilt/anxiety of choosing.

7. Practice assertive statements/role play for a realistic situation.
8. Set a small behavioral experiment in independent decision-making.
9. Explore cultural/family context respectfully.
10. Review what the client chooses to own as a next step.

Suggested mark allocation: Award marks for correct formulation/approach, risk and ethical priorities, logical session structure, appropriate technique application, collaboration/homework/follow-up, and professional communication. Credit clinically sound alternatives when clearly justified. Do not award full marks for naming a theory without showing how it is applied to the facts of the vignette.

Memory aid: FMRC + GSCR.

SECTION II

1. Retirement and loss of meaning - anchor: Existential therapy with screening for depressive disorder

After retiring from a long career, Fatou, 33, says, "Without my work I do not know who I am." she has become socially withdrawn, anxious about aging and preoccupied with whether life still has purpose.

Model response / marking points (25 marks):

1. Use empathic exploration of the transition and identity loss.
2. Assess depressive symptoms, functioning, medical issues and suicide risk.
3. Explore meaning previously obtained from work and other possible sources of meaning.
4. Discuss freedom and responsibility in shaping the next life chapter.
5. Acknowledge existential anxiety, aging, mortality and isolation without trying to erase them.
6. Clarify values and chosen commitments.
7. Identify relationships, contribution, creativity or experiences that can express those values.
8. Set one concrete action consistent with meaning/connection.
9. Avoid imposing the therapist's own life philosophy.
10. Review and follow up on both existential and clinical symptoms.

Suggested mark allocation: Award marks for correct formulation/approach, risk and ethical priorities, logical session structure, appropriate technique application, collaboration/homework/follow-up, and professional communication. Credit clinically sound alternatives when clearly justified. Do not award full marks for naming a theory without showing how it is applied to the facts of the vignette.

Memory aid: FMRC: Freedom, Meaning, Responsibility, Choice.

2. Irrational beliefs and self-worth - anchor: REBT

After a relationship breakup, George, 40, repeatedly says, "I absolutely must be loved by everyone. If someone rejects me, it proves I am worthless and I cannot stand it."

Model response / marking points (25 marks):

1. A: identify activating event - breakup/rejection.
2. B: identify rigid beliefs - must be loved, global self-downing, awfulizing, low frustration tolerance.
3. C: link beliefs to unhealthy emotion/behavior such as depression, panic, pleading or avoidance.
4. D: dispute logically, empirically and pragmatically - where is the evidence that rejection makes a whole person worthless?
5. E: develop effective rational beliefs - prefer love/approval but do not need universal approval; rejection is painful but tolerable.
6. Use rational emotive imagery or behavioral homework.
7. Distinguish healthy sadness from self-condemnation.
8. Set a practice task that tests new beliefs.
9. Monitor change in emotion and behavior.
10. Maintain collaborative but active style.

Suggested mark allocation: Award marks for correct formulation/approach, risk and ethical priorities, logical session structure, appropriate technique application, collaboration/homework/follow-up, and professional communication. Credit clinically sound

alternatives when clearly justified. Do not award full marks for naming a theory without showing how it is applied to the facts of the vignette.

Memory aid: ABCDE.